SACTCHECK · NCCP 00250

My complete treatment handbook

Docetaxel + cyclophosphamide (TC)

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 23
4. Supportive medicines and care — page 26
5. Visits, access and practical arrangements — page 27
6. Your treatment passport and symptom record — page 28
7. Wallet card and phone instructions — page 33

SACTCheck v0.84.33 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Docetaxel + cyclophosphamide (TC)

Chemotherapy after surgery to reduce breast cancer recurrence risk.

Adjuvant high-risk node-positive or node-negative operable breast cancer · NCCP 00250 v9 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

My overall treatment

Docetaxel and cyclophosphamide on Day 1 every 21 days, for four to six cycles. Confirm your planned number.

Chemotherapy
Both medicines at each treatment visit.

After chemotherapy
Radiotherapy follows if needed; agree further treatment and follow-up.

A typical 21-day cycle

Day 1–21
Your current reference cycle

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Blood count, kidneys and liver** Required by regimen	Baseline
During	**Repeat blood tests and symptom review** If clinically indicated	Team arranges tests; NCCP specifies repeats as clinically indicated
After	**Recovery and further treatment** Team-agreed plan	Individual appointments; no universal scan interval

Between treatments

- Report fever, sudden illness, worsening numbness or swelling.

Take docetaxel steroids as prescribed; reconcile with anti-sickness medicines.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

2 Blood / infection

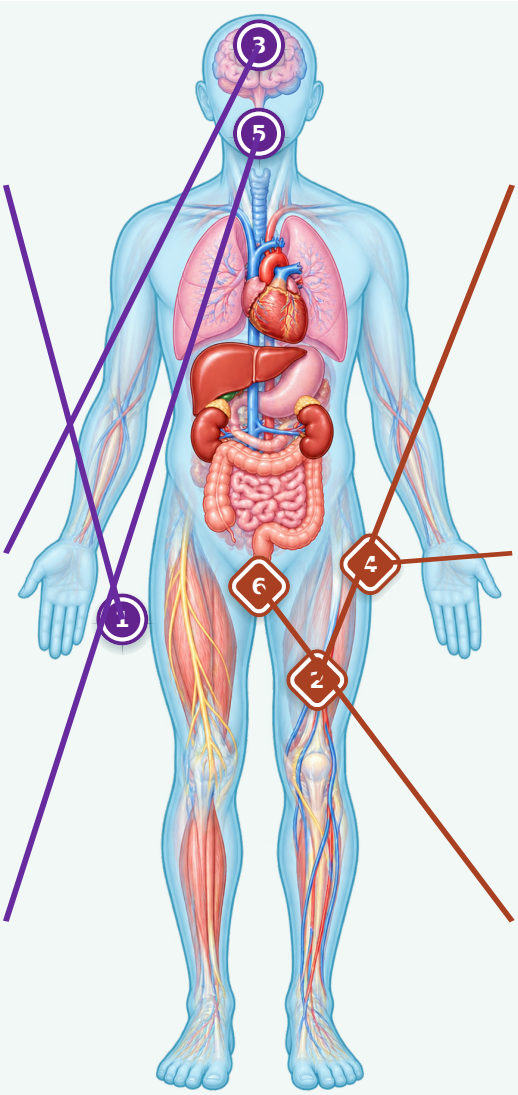
Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.



4 Fluid / skin / nails

Docetaxel

Report swelling, painful skin or nail changes.

5 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.

6 Bladder

Cyclophosphamide

Report blood in urine or painful urination.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Keep your treatment details to hand

Adjuvant high-risk node-positive or node-negative operable breast cancer · NCCP 00250 v9 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Docetaxel + cyclophosphamide

Both medicines on Day 1 every 21 days. Team confirms four or six cycles.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00250 · VERSION 9

Your treatment, explained

Adjuvant high-risk node-positive or node-negative operable breast cancer · NCCP 00250 v9 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Docetaxel + cyclophosphamide (TC)

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- The phases and how they connect
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Why this treatment?
- Checks across treatment
- Surgery, radiotherapy and later treatment
- Supportive medicines
- Trial results
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

How the medicines work

Treatment after surgery to reduce the chance of breast cancer returning.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell’s division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

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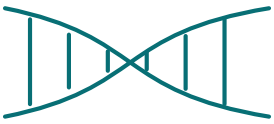
What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

Sources: nccp

ALKYLATING CHEMOTHERAPY

Cyclophosphamide



Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

The phases and how they connect

21-DAY INTERVAL

TC

4–6 cycles

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

The treatment phase determines the medicines, supportive prescription and tests. A phase change does not reset previous drug exposure or cardiac monitoring.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00250 v9 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel + cyclophosphamide (TC): treatment and checks

Adjuvant high-risk node-positive or node-negative operable breast cancer

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00250	Agree aim, indication and medicines	Docetaxel + cyclophosphamide (TC) starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan
Surgery EARLY	Adjuvant: surgery already completed	No automatic operation during cycles	Review postoperative recovery and symptoms	Surgery was completed before this adjuvant regimen	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00250	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00250 · ESC	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG NCCP 00250	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00250	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health NCCP 00250	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1

Before first dose

Baseline

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00250

2

Heart-monitoring plan

Baseline → cycles → after treatment

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00250 · ESC

3

Treatment-phase review

Before each transition

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00250

4

Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00250

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17

TC

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

TC

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Chemotherapy · Day 1	IV																				
Administration reaction	Report during or after any visit																				
Nausea / appetite	May occur early or later																				
Blood counts / infection	Risk varies; fever needs prompt advice																				
Fatigue / mouth changes	Variable timing and severity																				

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Nerves / skin / nails	Can build across doses																				

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

Phase changes: Do not change the schedule using this educational map.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00250 v9 · official regimen ↗️

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Read the whole cycle as text

Day 1–21: Your current reference cycle

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy. Follow the team's appointment and medicine instructions; contact them about missed doses or new symptoms.

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.



STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00250 v9 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 The correct phase prescription

TC uses moderate-risk antiemetic support.

2 Premedicines and growth factors

Docetaxel dexamethasone is taken for three days starting the day before treatment. Ask the team to reconcile this with antiemetic steroids.

3 Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

4 **After chemotherapy**

Discuss recovery, fertility, rehabilitation and the next treatment or surveillance step.

Sources: nccp · antiemetic

TREATMENT REFERENCE · 01

Why this treatment?

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Your treatment setting

Adjuvant high-risk node-positive or node-negative operable breast cancer

Sources: nccp

The sequence

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Sources: nccp

Alternatives and your priorities

Ask what alternatives fit your cancer, what the treatment aims to achieve, and how its practical demands fit your life. The team can discuss personal benefits and risks.

Sources: nccp

Checks across treatment

Your team arranges, reviews and acts on tests; an overdue result needs follow-up.

Blood tests

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Sources: nccp

Heart monitoring

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Sources: nccp

ECG

ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.

Sources: nccp

Cancer scans

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Sources: early

TREATMENT REFERENCE · 03

Surgery, radiotherapy and later treatment

Surgery → this adjuvant chemotherapy → radiotherapy if indicated. The surgical team decides lumpectomy versus mastectomy; the radiation plan depends on surgery, pathology and individual risk.

Hormone-receptor-positive disease

Endocrine treatment is considered when appropriate for the cancer biology and setting. It has its own schedule and monitoring; it is not automatically part of this chemotherapy prescription.

Sources: early · metastatic

Bone health

DEXA is linked to an aromatase inhibitor, ovarian suppression, treatment-induced menopause or other bone risk—not automatically to every chemotherapy course. The endocrine team confirms baseline testing and risk-based repeat scans.

Sources: bone

Markers and follow-up

After local treatment, the team plans mammography of remaining breast tissue. A mastectomy side does not need routine mammography. Report new symptoms rather than waiting for a scan.

Sources: bone

Genomic tests

Where appropriate, the MDT may discuss a validated genomic test to inform the chemotherapy decision. This regimen card does not calculate an Oncotype result or select treatment from a score.

Sources: early

UNDERSTANDING THE EVIDENCE

Why this treatment may be chosen

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Show trial results and what the numbers mean

Adjuvant high-risk node-positive or node-negative operable breast cancer

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit and alternatives in the context of your own cancer and priorities.

Sources: nccp · early

PREPARE FOR THE CONVERSATION

Questions worth asking

What changes between phases?

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

When are heart checks due?

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. Test timing follows the treatment, clinical findings and local pathway.

Where do surgery and radiotherapy fit?

In adjuvant care, radiotherapy usually follows chemotherapy when indicated; endocrine therapy follows its own receptor-specific plan.

Do I need routine CT scans or tumour markers?

Early breast follow-up uses planned breast imaging. Routine CT / PET, blood tumour markers or other distant-recurrence screening are not recommended without symptoms or a clinical indication. After local treatment: annual mammography of remaining breast tissue; additional imaging when indicated.

Does a port mean a pump at home?

No. This regimen has no routine take-home infusion pump. A port is an access device; the team removes the needle after the visit when appropriate and arranges device removal separately.

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

Why might this treatment be selected for me?

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

> **Try explaining the plan back:** Can you explain which medicines you are receiving now? What symptoms mean you should call urgently? Which blood tests, scans or heart checks come next, and who arranges them?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00250 v9. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00250 v9 · official regimen
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NICE NG101 · bone health and follow-up
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · implanted port patient information
- eviQ · PICC patient information
- NCCP NCIS antiemetics v8 (2025), Table 1
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · 2026
- Regimen selection and source references

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-10-23.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6353/250_DOCEtaxel-cycloPHOS_TC.pdf

https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext

https://academic.oup.com/eurheartj/article/43/41/4229/6673995

https://www.nice.org.uk/guidance/ng101/chapter/Recommendations

https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://ascopubs.org/doi/10.1200/JCO.2015.61.1459

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00250 v9 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel + cyclophosphamide (TC): treatment and checks

Adjuvant high-risk node-positive or node-negative operable breast cancer

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00250	Agree aim, indication and medicines	Docetaxel + cyclophosphamide (TC) starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan
Surgery EARLY	Adjuvant: surgery already completed	No automatic operation during cycles	Review postoperative recovery and symptoms	Surgery was completed before this adjuvant regimen	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00250	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00250 · ESC	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG NCCP 00250	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00250	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health NCCP 00250	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00250

2 Heart-monitoring plan

Baseline → cycles → after treatment

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00250 · ESC

3 Treatment-phase review

Before each transition

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00250

4 Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00250

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17

Docetaxel + cyclophosphamide (TC)

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Taxane / combination phase** — Moderate · cyclophosphamide with docetaxel
  Docetaxel + Cyclophosphamide

https://sactcheck.com/patient/00250/supportive-care.html

Docetaxel + cyclophosphamide (TC)

NCCP 00250 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Sources: NCCP 00250 v9 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

Docetaxel + cyclophosphamide (TC)

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00250/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Docetaxel + cyclophosphamide (TC)

Cycle number: _____ Start date: _____ Next review: _____

TC · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Cyclophosphamide · Alkylating chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Urinary symptoms	☐ Usual pattern	☐ Mild change: report	☐ Pain or frequent urination: call	☐ Cannot pass urine / marked bleeding: urgent review	☐ Collapse or severe illness: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Docetaxel + cyclophosphamide (TC)

Cycle number: _____ Start date: _____ Next review: _____

TC · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

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Dates / doses missed or changed / symptoms affecting daily life / questions:

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Recording sheet 2 · Continue on another sheet if needed · SACTCheck v0.84.17

Docetaxel + cyclophosphamide (TC)

Cycle number: _____ Start date: _____ Next review: _____

TC · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
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Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Docetaxel + cyclophosphamide (TC)

Cycle number: _____ Start date: _____ Next review: _____

TC · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

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Cyclophosphamide · Alkylating chemotherapy

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Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Adjuvant high-risk node-positive or node-negative operable breast cancer · NCCP 00250 v9 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Docetaxel + cyclophosphamide

Both medicines on Day 1 every 21 days. Team confirms four or six cycles.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00250 v9 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.